INSTITUTIONAL EQUITY RESEARCH / THESIS MONITOR

UnitedHealth Group (UNH)

Investment thesis refresh & portfolio monitoring brief

13 November 2026 · Refresh / Re-underwrite · Research evidence as of 13 Nov, market price and portfolio export close 12 Nov · Prior thesis 19 Oct 2026

RESEARCH POSTURE	UNH POSITION	LAST VERIFIED CLOSE	BASE DCF*
HOLD	**1.16%**	**$405.13**	**$470**
Watch for proof · 7/10 thesis confidence	50.22 shares · version 68	12 Nov 2026, not intraday	~16% modeled price upside

> **Bottom line.** UNH's recovery has better operating evidence than in early 2026, but reserve releases, membership contraction, and adverse medical-cost / regulatory exposure keep the sustainable earnings power unproven. Continue holding the existing modest position; do not accelerate additions at roughly $405 until reserve-adjusted underwriting and 2027 plan economics confirm the recovery.

Research judgment, not a Portfolio CIO-authorized trade instruction. The existing authoritative thesis status is preserved. The latest 10-Q, rather than a new earnings release, is the substantive disclosure since the October 19 assessment.

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01 / What changed since the October 19 underwriting?

Dimension	Then → now	Thesis impact
2027 expectations	Earlier analyst estimate about $22.61 adjusted EPS (October reference); current Yahoo consensus $23.33 for FY27 and $20.52 for FY26.	**Improving expectations**, not yet formal company FY27 guidance. Estimated 2027 EPS is ~3.2% higher than the earlier reference.
Valuation	19 Oct close $406.88 → 12 Nov verified close $405.13. Approximately 17.4× FY27 adjusted consensus EPS.	**Modestly more attractive** on consensus earnings; no meaningful price drawdown to create a deep margin of safety.
10-Q risk details	Form 10-Q filed 3 Nov details MA pricing pressure, medical-cost trend, Optum Health value-based care contractions, audits and unresolved litigation.	**More explicit risk disclosure**; largely confirms prior operating risks rather than a fresh collapse.
Capital allocation	10-Q reports board's November 2026 amendment allowing up to 50 million additional shares to be repurchased.	**Optional support** for per-share growth; an authorization is not a commitment to spend and does not cure underwriting quality.

02 / Primary-source financial evidence

Evidence supporting stabilization

- Q3 adjusted EPS **$4.69**; GAAP EPS **$4.56**; operating earnings **$6.56B** (+52% year over year).
- Q3 MCR **88.4%** versus **89.9%** a year earlier; administrative operating-cost ratio 12.8% versus 13.5%.
- 2026 adjusted EPS guide ≥ **$20.50**; Q3 operating cash flow **$5.1B**; first nine months operating cash flow **$25.1B**.

Evidence limiting confidence

- Consolidated Q3 revenue **$112.69B** versus $113.16B year earlier; UHC revenue down 1%; Optum Health down 10%.
- MCR included **$610M** of favorable development for earlier 2026 service periods: roughly **70 basis points** of quarterly premium revenue.
- Indicative MCR without that benefit ≈ **89.1%**, assuming dollar-for-dollar reversal; this is an analytical normalization, not a

- Optum Health operating earnings **$735M** versus $41M a year earlier (3.2% vs 0.2% margin).

company-reported measure.

- UHC served about **1.8M fewer people** year over year. Management cites above-historical medical-cost trends and MA funding gaps.

> **Quality-of-earnings test.** Directional margin improvement is real; the precise sustainable level is not. Reserve releases, business exits and restructuring gains can coexist with beneficial pricing action. Re-underwrite the core MCR and Optum margins without those items before accepting 2027 consensus as recurring earnings power.

03 / Thesis pillar scorecard

Pillar	What is working	What is weakening / unresolved	Research status
Insurance repricing	Q3 loss ratio and UHC operating profit improved; 2027 plan designs can prioritize adequate pricing.	Reserve normalization ~89.1% MCR; elevated commercial trend; MA funding below cost trend.	**Checkpoint**
Optum recovery	Optum Health 3.2% Q3 operating margin vs 0.2% prior year; Optum Insight and Rx operating earnings also advanced.	Optum Health revenue -10%, lower value-based covered lives; favorable development and portfolio actions contributed.	**Checkpoint**
Scale and cash	Broad health benefits, delivery, pharmacy, analytics footprint; 9M operating cash flow $25.1B versus $18.6B last year.	Structural leverage/regulation, claims reserve dynamics, ~$74.3B total short and long-term debt, debt-to-capital 41.7%.	**Working with caveats**
Growth durability	2027 analyst EPS consensus ~$23.33 implies ~13.7% growth versus ~$20.52 in 2026.	Flat Q3 group revenue, lost membership; can earnings grow sustainably without ongoing shrinkage or one-off benefits?	**Unproven**
Governance/regulation	Company disclosed reviews and risk controls; continued cash generation gives flexibility.	CMS RADV reviews, historical DOJ False Claims Act litigation, PBM and policy oversight, IRS transfer-pricing dispute.	**Persistent risk**

The strongest contrary case: favorable reserve development and market exits have pulled a declining-revenue franchise into temporarily better margins; 2027 benefit resets could lead to more membership losses and a weaker future earnings base than current consensus assumes.

04 / Valuation, DCF and reverse-DCF

At the 12 November verified closing price of **$405.13**, the stock is priced at ~**19.7× FY26** and **17.4× FY27** consensus adjusted EPS. The implied one-year EPS growth is ~13.7%. A mechanically computed ~1.27× one-year PEG is *not a robust normalized PEG*, because 2026–27 recovery makes near-term growth unusually sensitive to one-off factors. The Street 12-month mean target is approximately $476; it is a sentiment cross-check, not intrinsic value.

Fresh, explicitly illustrative 10-year equity DCF. Fiscal-2027 owner free-cash-flow-to-equity (FCFE) inputs are research assumptions, not company guidance. Forecast FCFE rises for ten years; terminal cash flow grows thereafter. Cash flows are discounted at the assumed equity required return and divided by **896.0M** shares. There is no separate debt subtraction in this FCFE framework; capital needs are reflected only to the extent the forecast captures them.

Scenario	2027 normalized FCFE	10-year growth	Cost of equity	Terminal growth	Value/share	vs. $405.13
Bear	$19.0B	6.0%	11.0%	2.5%	**$327**	-19%
Base	$20.5B	7.5%	10.0%	3.0%	**$470**	+16%
Bull	$22.0B	9.0%	9.5%	3.0%	**$611**	+51%

Reverse DCF: Using the base initial FCFE of $20.5B, 10% cost of equity and 3% terminal growth, the verified price implies roughly **5.5% annual FCFE growth** for ten years. The earlier October 19 reverse-DCF calculation (~4.7%) used a different inherited model and is **not strictly comparable**. A lower growth hurdle is not proof the company can meet it.

Sensitivity is significant: the modeled base is ~$470 and the bear ~$327. DCF assumptions have not been audited against a full independent FCFE bridge (insurance working capital, statutory capital, acquisitions and loan flows). Treat the range as scenario discipline, not an exact target. The previous October 19 FCFE DCF base of $476 is retained as historical research, not silently overwritten by identical assumptions.

05 / Portfolio fit — verified canonical version 68

Shares	Unit cost	Total cost basis	Snapshot market value	Unrealized price gain	Portfolio weight
50.22	$237.45	$11,924.74	$20,345.63	+$8,420.89 (+70.6%)	1.163%

Sole holdings and basis source: **retirement-portfolio-68.xlsx**, SHA-256 `5cabd71bf23b5dbe35489e89255fd4cc5e298d049712f9c988dab6c3c6c03c50`, matched to portfolio manifest. The workbook price of $405.13 is consistent with the 12 Nov public close; the 1.16% weight refines the rounded 1.2% user description. No linked accounts or earlier exports were used.

Research portfolio posture: A 1.16% holding provides a reasonable option on recovery without an outsized single-company regulatory risk. Existing mark-to-cost gains are not evidence of future value. Keep position while awaiting cleaner 2027 evidence; under a CIO-approved capital-allocation process, a **$365–$385** staged-add area merits consideration only if current underwriting evidence remains intact. Below ~$350, reassess the reason for the decline before buying. Around ~$450+, absent material upgrades, margin of safety is narrower. Incremental-add conviction: **6/10**.

06 / Upcoming checkpoints — specific evidence and actions

When	What to measure	Favorable confirmation	Concern / action
Through 7 Dec 2026	2027 Medicare Advantage open enrollment, local plan and network retention, star-rating mix.	Retention in targeted profitable geographies; stable star-bonus economics.	Disproportionate exits/weak appeal in profitable counties → review 2027 EPS assumptions.
Q4 2026 results; likely Jan 2027 (date not company-confirmed)	GAAP vs adjusted EPS, MCR, reserve development, Optum Health margin, Q4 cash flow; FY27 formal guide.	Core reserve-adjusted MCR on improving trajectory; FY27 adjusted EPS credible around $23+, supported by margin bridge.	Another reserve-led beat, FY27 guide below $22 or weak Optum unit economics → suspend adds / re-underwrite.
Jan–Feb 2027	CMS MA membership data after new-plan enrollment and 2027 pricing; county/product mix.	Membership losses moderate as profitable books stabilize.	Large additional exits without commensurate margin gains → revise revenue and persistence assumptions.
Q1 2027 print	First 2027 earned premium/medical cost test; Optum margins excluding favorable reserve release.	Reserve-adjusted insured loss ratio improving toward ≤88.5%; Optum Health margin consistently ≥3%.	Core MCR ≥89.5% with weak Optum margins → escalate to CIO for thesis-break review.

When	What to measure	Favorable confirmation	Concern / action
Continuous / on formal action	DOJ False Claims litigation, CMS RADV, regulatory capital, PBM remedies, IRS proceedings.	No material permanent adverse economics.	Quantified impairment of recurring economics triggers prompt event-driven underwriting.

07 / Explicit thesis-break conditions

All conditions below are **draft research falsifiers for Portfolio CIO approval**, not automatically executable trading instructions. Confirmation should use official reports and distinguish one-time seasonality from recurring deterioration.

1. **Underwriting failure:** two successive comparable quarters of core, reserve-adjusted UnitedHealthcare MCR **≥89.5%** despite 2027 benefit repricing, with no credible benefit from seasonality or member mix.
2. **Recovery earnings broken:** formal 2027 adjusted EPS guidance **<$21.00**, or a ≥10% downward revision driven by repeat medical-cost underestimation and absent credible corrective pricing.
3. **Optum Health thesis damaged:** operating margin **≤1%** in two successive quarters, excluding identifiable nonrecurring gains and favorable reserve changes, plus evidence the economics of accountable care remain structurally impaired.
4. **Structural regulation/coding loss:** a binding adverse DOJ/CMS/RADV or PBM outcome that permanently reduces normalized earnings materially (illustrative threshold: **≥$5B** of after-tax economic value or ~100bp sustainable company margin, if substantiated). Ongoing investigations alone do not meet this test.
5. **Franchise contraction without economic offset:** 2027 MA membership declines by another **~1M+ members** relative to end-2026 while consolidated operating margins fail to improve despite exits and repricing.

Escalation before a formal break: rolling annual operating cash conversion <0.8× net income together with adverse reserve development; sustained debt-to-capital above ~45% to fund distributions; or repeated guide cuts. A single such flag is a high-priority re-underwrite, not automatically a sale.

08 / Investment conclusion, uncertainty and governance

Company Quality: High strategic scale, but earnings-quality and policy execution temporarily reduce confidence. **Valuation Attractiveness:** Moderately attractive (~17.4× FY27 adjusted EPS, illustrative DCF bear/base/bull ~$327/~$470/~$611); meaningful downside remains. **Portfolio Fit:** Reasonable at 1.16%; concentration and potential future increases require Portfolio CIO approval.

Research conclusion — Hold / wait for proof (7/10 confidence in an improving but incomplete thesis). Q3 demonstrated operating progress, and later consensus revisions are encouraging; however, the November filing reinforces that higher medical-cost intensity and smaller covered populations are structural hurdles. The next investable signal is a clean FY27 earnings bridge with reserve-adjusted MCR and Optum economics, not simply another EPS beat. **No authoritative CIO thesis-status change or trade is executed.**

Unanswered diligence: true recurring MCR after reserve runoff; FY27 cohort pricing by geography; Optum Health profitability by risk cohort excluding divestitures; cash tied up in required capital and loan flows; potential economic exposure under active audits/tax disputes.

Source notes and citations

1. UnitedHealth Group, Q3 2026 earnings release, 13 Oct 2026; SEC earnings schedules.
2. UnitedHealth Group, September 2026 10-Q (filed 3 Nov 2026), especially operating review, claims/reserves, risk, cash-flow and buybacks.
3. Yahoo Finance analyst estimates, accessed 13 Nov 2026 (FY26 EPS $20.52 and FY27 EPS $23.33); price history, 12 Nov 2026 close $405.13. Estimates from third-party aggregation are not management guidance.
4. CMS 2027 MA/Part D Star Ratings, 8 Oct 2026. 2027 quality bonuses generally affect 2028 payments.
5. Portfolio source: **retirement-portfolio-68.xlsx** (private uploaded workbook, validated SHA-256 above); internal Portfolio-Summary UNH thesis record last re-underwritten 19 Oct 2026. No earlier holdings file used.

Research prepared 13 November 2026. Modeling assumptions are scenario estimates and exclude specific unknown adverse regulatory settlements. Forecast outcomes are uncertain. Distribution: private analysis; publication to Portfolio Command Center requires separate explicit approval.